



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : نجاه زكي مرقص ابراهيم
المحافظة : القاهرة
الحى / المدينة : النزهه
العنوان : النزهه
رقم البطاقة : 26002020100801 رقم التليفون : 01228474160
العمر (تاريخ الميلاد) : 02-02-1960 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 03-10-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام رنين مغناطيسي علي المخ والحجاج بالصبغه
[11112]
قسم العلاج :
الأورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات بالمرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. ----- /د. ----- /د. ----- /د. -----

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: نجاه زكي مرقص ابراهيم National ID: 26002020100801	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced MRI of the brain and orbits
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تعريف شخصية

نجاه
زكى مرقص ابراهيم
٣ صمات النصر - مساكن شيراتون
النزهة - القاهرة

١٩٦٠/٠٢/٢٢ ٢٦٠٠٢٠٢٠١٠٠٨٠١
KP9123880

٢٠٢٤/٠٦ ٢٦٠٠٢٠٢٠١٠٠٨٠١
حاصلة على دبلوم تجارة
أنثى مسيحية أرمنية
البطاقة سارية حتى ٢٠٣١/٠٦/٢٥ ١

٢٠٢٤/٠٦ ٢٦٠٠٢٠٢٠١٠٠٨٠١



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



A-Patient ID:

1- Name : نجاء زكي مرصص ابراهيم

SEX:

MDT form

3- Medical number:
Female

83163786

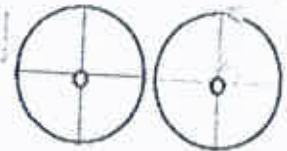
Old Med :

Old Med :

3534-2023

2-ID number: 25002020100801

Patient Age : 64 Y



Family history

Family History
+ve

Phone

Phone
01223292681

BMI

BMI

REASON OF MDT

REASON OF MDT
POST OPERATIVE

marital status

Marital Status

NRS Score

NRS Score

GROUP

GROUP

Date of MDT

Date of MDT

2023-11-11

Co morbidity

Co Morbidity

Menopausal
status

Menopausal Status
POSTMENOPAUSAL

Clinical data

Breast Imaging :

left OUQ and retro areolar suspicious masses
+ Ipsilateral axillary pathological lymph . nodes BIRADS 4C

Meta Workup :

CT (contrast) CAP
Insignificant tiny calcification at hepatic dome .
otherwise normal CT scan

BX
IDC

ER
8/8

PR
0/8

Her 2 Neu
0

SISH

KI67
45

BX COMMENT:

ER COMMENT:

PR COMMENT:

Her 2 Neu COMMENT:

SISH COMMENT :

T
2

N
2

M
0

Sonomammography

PATIENT HISTORY

RT MRM:

free margins

LN 7/17

MDT OUTCOME & PLAN

FOR ADJUVANT 4C AC/ 4C TXT THEN PORT, AI 5 YS
DEXA SCAN, PROLIA/6 MNS FOR 3 YS AFTER DENTAL

SIGNATURE

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

RADIOLOGY

RADIOLOGY

SURGERY

SURGERY

PATHOLOGY

PATHOLOGY



Patient Name : الأستاذة / نجاة زكى مرقص ابراهيم
Patient No. : 26002020100801
Path No. : 3661/23
Sex : Female
Age : 63 Y
Referring Doc :

Request No. : 13007439
Request Date : 23/10/2023
Verification Date : 31/10/2023
Financial Category : أورام دار السلام - ميانره
Department : Out Patient
Registered by : .marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer

Nature of the specimen: Left breast & Left axilla (MRM)

Gross Examination:

- Procedure: Left MRM
- measured 17x15x5 cm with covering skin ellipse measured 16x11 cm .
- Localization: UOQ
- Tumor extent : A greyish white mass
- Size: 2.5 x2 x1.8 cm ,2nd mass 1x1 cm.
- Skin involvement: Nipple & areola were unremarkable grossly .
- Distance from margins: 1.5 cm away from deep margin , 0.4 cm away from the skin .
- Microscopic examination:
- Histological type: Multicentric Invasive duct carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis): G II(3+2+1)
- Ductal carcinoma in situ: Focal (10%), solid, papillary type
- Skin involvement: Free nipple and areola
- Lymph node involvement: Positive L.Ns (7/17) with extracapsular extension
- Resection Margins: Free margins
- Additional pathological findings: Duct papillomas, with duct ectasia

Diagnosis:

Left MRM (post NACT) :

Multicentric Invasive duct carcinoma grade II

- Free margins
- Free nipple and areola
- Positive L.Ns (7/17) with extra capsular extension
- Stage : pT2(m) , pN2

Verified by :

Dr.Heba Hasan

Printing Date : 04/11/2023 09:51:31

Page 1 of 2

Printing By : .marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name	: الأستاذ / نجاة زكي مرقص ابراهيم	Request No.	: 13007439
Patient No.	: 26002020100801	Request Date	: 23/10/2023
Path No.	: 3661/23	Verification Date	: 31/10/2023
Sex	: Female	Financial Category	: أورام دار السلام - مبانره
Age	: 63 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Signature
Prof. Dr. Amal Hareedy
Dr.Heba Hassan

Verified by :
Dr.Heba Hasan

Printing Date : 04/11/2023 09:51:31

Page 2 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذة / نجاة زكي مرقص ابراهيم
 Patient No. : 26002020100801
 Path No. : 2954/23
 Sex : Female
 Age : 63 Y
 Referring Doc. :

Request No. : 13003408
 Request Date : 02/09/2023
 Verification Date : 13/09/2023
 Financial Category : أورام دار السلام - مبداء
 Department : Out Patient
 IHC No. :
 Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast retroareolar mass
 Nature of Specimen: Tru-cut biopsy

Technique:

- Multiple fragmented greyish white tissue cores up to 1.5 cm, totally submitted.
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER, PR, HER2 and Ki67

Results:

- IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8).
- PR: No nuclear reaction in tumor cells (Allred score 0/8).
- Her2: No membrane reaction in tumor cells (score 0).
- Ki67: 45% nuclear reaction of tumor cells.

Conclusion:

Left breast retroareolar mass; Tru-cut biopsy:
 Invasive duct carcinoma, Grade III
 ER: Positive (score 8/8)
 PR: Negative (score 0/8)
 HER2 : Negative (score 0)
 Ki67: Positive in 45% of tumor cells.

Signature
 Dr. Rana Fathy

Verified by :
 Dr. Heba Hasan

Printing Date : 16/12/2023 14:17:07

Page 1 of 2

Printed By : Ali mostafa



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name	الأستاذة / نجاة زكي مرقص ابراهيم	Request No.	13003408
Patient No.	26002020100801	Request Date	02/09/2023
Path No.	2954/23	Verification Date	13/09/2023
Sex	Female	Financial Category	اورام دار السلام - مبادره
Age	63 Y	Department	Out Patient
Referring Doc.		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Verified by :
Dr.Heba Hasan

Printing Date : 16/12/2023 14:17:07

Page 2 of 2

Printed By : Ali mostafa



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذة / نجاة زكي مرقص ابراهيم	Request No : 13003408
Patient No. : 26002020100801	Request Date : 02/09/2023
Path No. : 2954/23	Verification Date : 07/09/2023
Sex : Female	Financial Category : اورام خارج السلام - مياثره
Age : 63 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Left breast retroareolar mass.

Nature of Specimen: Tru-cut biopsy, collected at 31/08/2023, 03:55 pm.

Gross Examination:

- Multiple fragmented grayish white tissue cores up to 1.5 cm, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+3+3) Grade III.
- * Ductal carcinoma in situ: Focally detected
- * Perineural invasion: Negative
- * Angioinvasion: Negative

Diagnosis:

Left breast retroareolar mass, Tru-cut biopsy:

Invasive duct carcinoma, Grade III.

For further IHC (ER, PR, Her2, Ki67).

Signature

Prof. Dr. Amal Hareedy

Dr. Heba Hassan

Verified by :

Dr. Hebatallah Mostafa

Printing Date : 14/09/2023 12:55:03

Page 1 of 1

Printing By : Ali mostafa

Medical Number: 672771

Age: 64 Years

Patient Name: نجاة زكي مرقص ابراهيم

Gender: Female

Date of Exam: 18/09/2023 10:58:43 AM

Diagnosis: -----

POST CONTRAST CT SCANS OF CHEST, ABDOMEN AND PELVIS

- Both lungs are clear with no ground glass opacities, consolidations, masses or nodules.
- No pathologically enlarged hilar or mediastinal lymph nodes.
- Patent tracheo-bronchial tree.
- No pleural effusion or pneumothorax.
- Liver is normal in size and homogenous in density with no focal lesions or pathological contrast enhancement. A tiny calcification is seen at its dome. No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of spleen, kidneys, pancreas, adrenals and urinary bladder.
- Previous hysterectomy with no pelvic masses.
- No abdominal lymphadenopathy.
- No ascites or fluid collection.

Conclusion:-

- Left breast retroareolar irregular mass-lesion with thickened overlying skin and left axillary lymphadenopathies.
- Insignificant tiny calcification at hepatic dome, otherwise, normal CT scans of chest, abdomen and pelvis.

Much Obligated,

KHALED SHEHATA, MD.





Name : Mrs. Nagat Zaky Morcos Abraham

Code : 3069666

Date : 11/06/2024

Tc-99m MDP BONE SCAN:

Procedure:

- Whole body skeletal survey was obtained 3 hours post IV injection of 21 mCi of the tracer.

Findings:

- No abnormal focal active or cold osseous lesions could be detected to account for bone metastases
- Degenerative changes all over the spine are noted, being exaggerated at the lower lumbar vertebrae
- Osteo-arthritic changes of shoulders , knees and ankles are also observed.

Opinion:

- The skeleton is free from metastases.

Best Regards,
Prof. Dr. Maha Abd Elkareem
Professor of Nuclear Medicine
kasr al ainy, cairo University

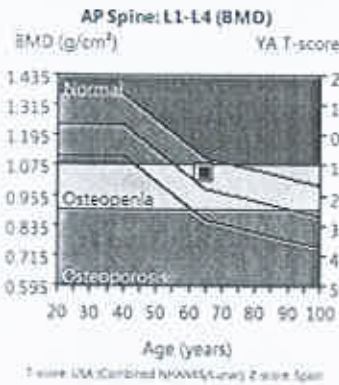
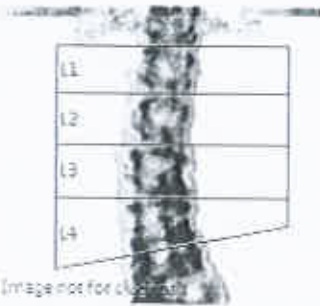
For any inquiries regarding the findings in the report please contact us on
radreport@cairoscan.com.eg

Patient: naga zaki moroks
Birth Date: 02/02/1960
Height: 150.0 cm
Sex: Female

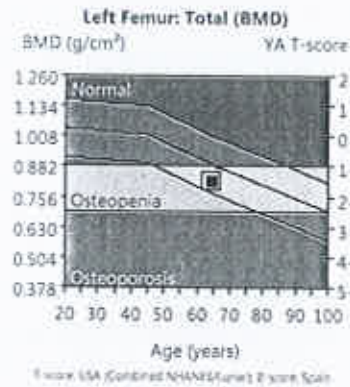
Age: 64.3 years
Weight: 70.0 kg
Ethnicity: Hispanic

Referring Physician: (not specified)

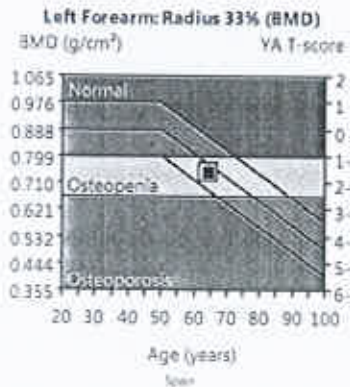
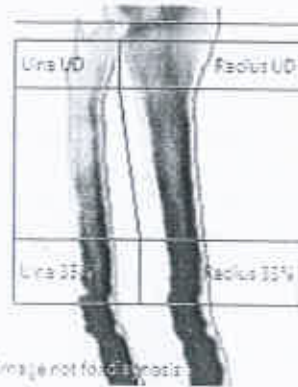
Patient ID: (not specified)
Measured: 22/06/2024 10:25:37 ص (16 [SP 1])
Analyzed: 22/06/2024 10:33:24 ص (16 [SP 1])



Region	BMD (g/cm ³)	YA (%)	YA T-score	AM (%)	AM Z-score
L1	0.932	82	1.7	100	0.0
L2	0.956	79	2.1	95	0.4
L3	1.081	89	1.1	107	0.6
L4	1.169	96	0.4	116	1.3
L1-L2	0.945	80	1.9	98	0.2
L1-L3	0.998	85	1.5	102	0.2
L1-L4	1.045	88	1.2	106	0.5
L2-L3	1.025	85	1.5	102	0.2
L2-L4	1.075	89	1.1	107	0.6
L3-L4	1.124	93	0.7	112	1.0



Region	BMD (g/cm ³)	YA (%)	YA T-score	AM (%)	AM Z-score
Neck	0.820	79	1.6	99	0.0
Upper Neck	0.693	84	1.1	-	-
Wards	0.677	74	1.8	102	0.1
Shaft	0.975	-	-	-	-
Total	0.822	82	1.5	92	0.6



Region	BMD (g/cm ³)	YA (%)	YA T-score	AM (%)	AM Z-score
Radius UD	0.400	85	1.5	98	0.2
Ulna UD	0.253	-	-	-	-
Radius 33%	0.742	84	1.6	96	0.3
Ulna 33%	0.682	-	-	-	-
Both UD	0.349	-	-	-	-
Both 33%	0.713	-	-	-	-
Radius Total	0.573	84	1.8	95	0.5
Ulna Total	0.473	-	-	-	-
Both Total	0.531	-	-	-	-

Statistically 68% of repeat scans fall within: 1SD (± 0.010 g/cm³ for AP Spine L1-L4) (± 0.012 g/cm³ for Left Femur Total); (± 0.020 g/cm³ for Left Forearm Radius 33%); T-score: USA (Combined NHANES (ages 20-30) / Lunar (ages 20-40)) AP Spine: Female; White Reference Population (v113); T-score: USA (Combined NHANES (ages 20-30) / Lunar (ages 20-40)) Femur: Female; White Reference Population (v113); Spain (ages 20-40) Forearm: Female Reference Population (v113); Z-score: Spain AP Spine Reference Population (v113) Matched for Age, Sex, Weight (females 25-100 kg); Ethnic: Z-score: Spain Left Femur Reference Population (v113) Matched for Age, Sex, Weight (females 25-100 kg); Ethnic: Left Forearm Matched for Age, Sex, Ethnic; Lunar calibration in use; World Health Organization - Definition of Osteoporosis and Osteopenia for Caucasian Women: Normal = T-score at or above 1.0; SD; Osteopenia = T-score between 1.0- and 2.5- SD; Osteoporosis = T-score at or below 2.5- SD; (WHO definitions only apply when a young healthy Caucasian Women reference database is used to determine T-scores.)
Date created: 22/06/2024 10:33:33:15 ص (SP 1); Filename: NQHS3394.mrc; AP Spine: 76.15015304339.0001.00-0.60x1.20 22.2%fat+50.6% 0.000.00 0.000.00; Scan Mode: Standard; OneScan: 20.0 µGy; Left Femur: 76.15015304339.0001.00-0.60x1.20 19.5%fat+42.9% 0.000.00 0.000.00; Neck Angle (deg): 48; Scan Mode: Standard; 20.0 µGy; Left Forearm: 76.053426157.0001.00-0.60x1.20 5.8%fat+50.6% 0.000.00 0.000.00; Forearm Length: 28.6 cm; Scan Mode: Standard; 3.0 µGy



Name : Nagat Zaky Morcos Date 6-9-2026 SD
Dear Prof Dr.: Medhat Mishil

MRI OF THE BRAIN

FINDINGS:

- No areas of diffusion restriction seen in DWI.
- Few bilateral tiny cerebral foci of abnormal high T2 and FLAIR signal intensity are seen within the periventricular and subcortical white matter zones of both cerebral hemispheres. They are iso to hypointense in T1 WI, and exert no mass effect on the surrounding structures.
- Normal parenchymal signal intensity pattern of the cerebellum and brainstem.
- Slightly prominent ventricular system and extra-axial CSF spaces namely the cortical sulci, basal cistern and sylvian fissures.
- No shift of the midline structures.
- No extra-axial collections.
- Normal cranio-cervical region.

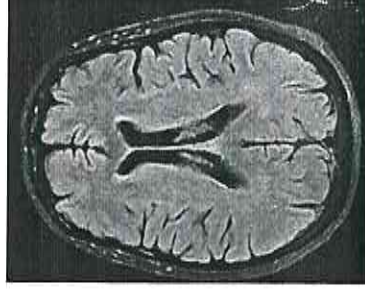
OPINION:

- No recent infarction or hematoma.
- Mild small artery disease with bilateral few tiny ischemic foci.
- Mild brain involutional changes, more cortical.

Truly your's

Hassan G.Mourad,M.D.

Mohamed Mourad,M.D. Hesham Atef. M.D.



Ref. by Prof. Dr Abdallah Hassouna

Date 26/09/2026

إسم المريض نجاه زكي مرقص ابراهيم

رقم الملف 1534181

Approved on: 26-09-2026 10:42 PM

Ultra-widefield OCTA

Dear Sir,

Thank you for referring this patient for ultra-widefield OCT Angiography assessment of both maculae

Angiography Analysis: Angiography 6x6 & 29x29 mm

Right eye

- Full thickness map topography shows decreased retinal thickness within the macular area
- Vitreoretinal interface (VRI) slab shows no abnormal flow.
- Superficial slab shows within normal vascular flow and within normal capillary density of Superficial Capillary Plexus (SCP).
- Deep slab shows within normal vascular flow and within normal capillary density of Deep Capillary Plexus (DCP).
- Foveal avascular zone (FAZ) looks circular with normal size.
- Outer retinal Layers (ORL) slab shows no abnormal flow.
- Choriocapillaris (CC) slab shows homogenous granular appearance.
- Corresponding B scan passing through the fovea shows multiple choroidal lesions with large temporal choroidal mass with overlying PEDs and SRF accumulation (mostly metastatic for further evaluation) otherwise dry macula with preserved foveal contour.

Left eye

- Full thickness map topography shows decreased retinal thickness within the macular area
- Vitreoretinal interface (VRI) slab shows no abnormal flow.
- Superficial slab shows within normal vascular flow and within normal capillary density of Superficial Capillary Plexus (SCP).
- Deep slab shows within normal vascular flow and within normal capillary density of Deep Capillary Plexus (DCP).
- Foveal avascular zone (FAZ) looks circular with normal size.
- Outer retinal Layers (ORL) slab shows no abnormal flow.
- Choriocapillaris (CC) slab shows homogenous granular appearance.
- Corresponding B scan passing through the fovea shows dry macula with preserved foveal contour and RPE irregularities.

Please correlate with FFA, OCT findings and clinical examination notes.

Sincerely

Madonna Mimi (MSc)

للمزيد من المعلومات
for more information

فرع الثورة :
إمتداد شارع الثورة، مصر الجديدة، أمام خوري الجيش، القاهرة.
فرع الحجاز :
11 شارع الحجاز، مصر الجديدة، القاهرة.
فرع الغردقة :
طريق المطار، الغردقة، البحر الأحمر.

PET Scan

[DOCTYPE:PET_SCAN]

Name	Mrs. Nagah Zaki Morkos Ibrahim		
Ref. By Prof. Dr.	Abdullah Kamel Hasona	Date	30/9/2026
ID	9926018097	Acc. No.	240343618



- No biliary tree dilatation.
- Average size of the spleen with no focal active lesions.
- Normal appearance of pancreas (no sizable active masses) and adrenals.
- Anterior abdominal wall and underlying fat planes low activity
- Reticulations / fibro-reticular densities of no evident uptake, mostly post management sequelae, however, for follow up.
- No enlarged FDG hypermetabolic abdominal or pelvic L.N.s seen.
- Congested mesentery with strandy reticulations.
- Normal physiological biodistribution over the bowel (non-uniform bowel uptake with no sizable masses) & urinary tracts.

Surveyed skeleton:

- D10 metabolically active mixed lytic / sclerotic osseous lesions; highest activity recorded of SUVmax. 9.
- Scattered sclerotic non-metabolically active foci of the spine and pelvic bones (mostly degenerative changes).

Opinion:

- Active pulmonary, nodal, hepatic and osseous lesions (metastatic deposit).
- For mammographic correlation.

Thanks for your referral;

Dr. Islam Aboulwafa, MD

H.S 01/10/26 4:52:58 PM

Consultant Radiologist

Name	Mrs. Nagah Zaki Morkos Ibrahim		
Ref. By Prof. Dr.	Abdullah Kamel Hasona	Date	30/9/2026
ID	9926018097	Acc. No.	240343618

¹⁸F-FDG PET/CT Scan

Indication of the study: Status evaluation study for a case with previous history of breast neoplasm. no available previous studies or pathological data.

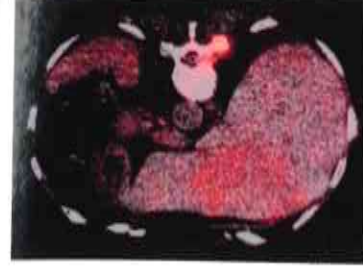
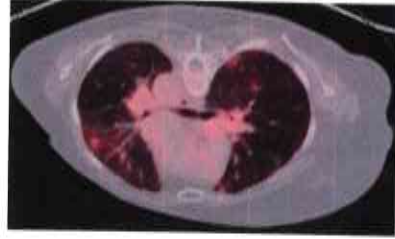
Head & Neck:

- Normal physiological biodistribution over scanned portion of the brain.
- (N.B; not all brain metastatic lesions could be detected on ¹⁸-FDG PET/CT due to physiologic uptake in brain. MRI brain is suggested in cases of clinical suspicion).
- Normal naso-/oro-pharynx, larynx, thyroid and salivary glands.
- No enlarged FDG hypermetabolic cervical L.N.s seen.

Torso:

Thorax:

- Low activity management bed reticulations.
- No focal breast active masses.
- No enlarged FDG hypermetabolic mammary related lymph nodes on the left side.
- Active right axillary lymph node about 1 cm of SUVmax. 4.
- For mammographic correlation.
- Active bilateral hilar and mediastinal (all groups) lymph nodes, largest about 3 cm of SUVmax. 7. Bilateral variable sized metabolically active pulmonary nodules of predominant subsolid configuration, largest measuring ± 1.2 cm, eliciting SUVmax. 4.8.
- No pleural or pericardial effusion seen.



- Average size of the liver with FDG hypermetabolic focal lesions, largest about 7 cm of SUVmax. 8. For dynamic MRI study assessment.

Abdomen & Pelvis:

Total body PET scan was done 40 minutes after IV administration of 300 MBq of ¹⁸F-FDG.

CT scanning was performed simultaneously without IV contrast material injection.

Semi-quantitative analysis would be done for selected ROIs & the normal threshold would be the normal blood pool liver SUVmax < 2.3